From: Adam Clutterbuck <ac@hxc.uk>
To: Nicholas Scarr <nscarr@quantlogic.com>
Date: Saturday 15 November 2025 at 17:18
Subject: RE: Qlaim1816 - 1 Riverside Cottages, The Dean, Alresford SO24 9BE

Hi Nicholas

Thank you for the additional information; I have to go out now for the eve but will drop back onto this in the morning to insert additional photos, and check for typos etc, as well as a proper review.

If you could please review my initial draft, and 4 No. appendices, and let me know of any inaccuracies, and/or of any additional information/amendments you would like me to consider for inclusion I will of course review this too.

My aim is to ensure you have a final document tomorrow that you can submit to insurers.

Kind regards

Adam

_____

Adam Clutterbuck

[A picture containing brass knucks, scissors Description automatically generated]

Home & Commercial

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T: 01722 714455 (direct line)

M: 07904 093727

From: Nicholas Scarr <nscarr@quantlogic.com>

Sent: 15 November 2025 16:00

To: Adam Clutterbuck <ac@hxc.uk>

Subject: Fwd: Qlaim1816 N Scarr - Detailed understanding of the Biotechnical position relating to 1 Riverside Cottages, The Dean, Alresford SO24 9BE

Hi Adam,

Please see a response we finally got from Bio Response yesterday.

It was not in response to the question regarding detailed activity report and as you will see it provides no results but in the interests of completeness I am sending it to you.

I did not send it immediately as it deals extensively with the contents which were not relevant to your request and the one we passed on to Bio Response. That said there are some dates you will find useful.

The Flood took place on the 28th February and it was reported to insurers on our return from holiday which was the 4th March. The claim may not have been reported until the 5th March as we were very preoccupied with dealing with the mess when we got back.

Kind regards,

Nicholas

Nicholas Scarr

CEO – Quantlogic

UK (M):+44 (0)7768 604849

Email: nscarr@quantlogic.com<mailto:nscarr@quantlogic.com>

Website: www.quantlogic.com<http://www.quantlogic.com/>

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---------- Forwarded message ---------

From: Darren Southwell <darren@bioresponse.co.uk<mailto:darren@bioresponse.co.uk>>

Date: Fri, 14 Nov 2025 at 11:09

Subject: Re: Qlaim1816 N Scarr - Detailed understanding of the Biotechnical position relating to 1 Riverside Cottages, The Dean, Alresford SO24 9BE

To: Andy Lyne <Andy.Lyne@qlaims.com<mailto:Andy.Lyne@qlaims.com>>, Bobby Bell <bobby@bioresponse.co.uk<mailto:bobby@bioresponse.co.uk>>

Cc: Claims <Claims@qlaims.com<mailto:Claims@qlaims.com>>, Nicholas Scarr <nscarr@quantlogic.com<mailto:nscarr@quantlogic.com>>

Good morning all

please see my comments inserted in red below

should you have any queries please do not hesitate in contacting me

kind regards

Darren Southwell Tech.Dew BDMA Tech

Insurance Department Manager

Bio Response Ltd

14 Fairways Business Centre, Airport Service Road, Portsmouth P03 5NU

Office: 0333 202 6416 Mo: 07585117609

Email: darren@bioresponse.co.uk <mailto:darren@bioresponse.co.uk>
<mailto:darren@bioresponse.co.uk>Web: www.bioresponse.co.uk<http://www.bioresponse.co.uk/>

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On Thu, Nov 6, 2025 at 7:32 AM Andy Lyne <Andy.Lyne@qlaims.com<mailto:Andy.Lyne@qlaims.com>> wrote:

Morning Darren,

I hope you're well.

Further to your recent meeting with Mr Scarr, which I attended via telephone, please see Mr Scarr's observations below.

I have read Mr Scarr's observation, and they are in order with my recollection of the discussion.

Best Regards

Andy Lyne

Qlaims Adjuster

Mob: +44 (0) 7887522332

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From: Nicholas Scarr <nscarr@quantlogic.com<mailto:nscarr@quantlogic.com>>

Sent: 04 November 2025 19:15

To: Andy Lyne <Andy.Lyne@qlaims.com<mailto:Andy.Lyne@qlaims.com>>; Claims <Claims@qlaims.com<mailto:Claims@qlaims.com>>

Cc: Claims <Claims@qlaims.com<mailto:Claims@qlaims.com>>; Louisa Scarr <scarrlouisa@gmail.com<mailto:scarrlouisa@gmail.com>>

Subject: Detailed understanding of the Biotechnical position relating to 1 Riverside Cottages, The Dean, Alresford SO24 9BE

Dear Darren,

Further to our meeting at BioResponse's offices last Thursday, which I attended in person (with Andy Lyne joining by telephone), I would like to record my understanding of the factual and professional points discussed in relation to 1 Riverside Cottages.

Please review the summary below and confirm that it accords with BioResponse's professional understanding. If you consider any part inaccurate, please identify it and set out the technical basis for your view.

Your firm indicated from the outset that this incident needed to be dealt with expeditiously to avoid significant secondary damage. Alas, this did not happen, amplifying the extent of secondary damage.

The delay did not alter the fundamental classification of the incident — sewage ingress is Category 3 from the moment of contact — but it did allow extensive humidity and cross-contamination to develop, exacerbating the depth of microbial infiltration. The property has remained, and continues to remain, in a contaminated condition pending comprehensive remediation and independent post-remediation verification, and is at present certainly far from any standard that could be described as hospital-grade.

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1. Chronology and Extent of Contamination

* The primary sewage ingress occurred on 28 February 2025, affecting both the interior of the dwelling and the front and rear external areas - The loss date on Davies portal has the loss date as 01/03/2025 and that the claim was recieved by them from the insurers on 05/03/2025 The initial instruction to us was recieved on 05/03/2025 following which an initial visit was arranged with yourself for 08/03/2025.

* The property remained in a contaminated condition for an extended period while investigations and drying operations were arranged. - The report and scope of works was completed and submitted on 11/03/2025 awaiting approval. We recieved approval from davies on 14/04/2025, thereafter we were awaiting for the stripping out works to be completed. On 29/04/2025 a query was raised with Elizabeth turner at Davies in relation to packing and removing unaffected items to storage as this is usually dealt with via another contractor. Following this a VO was submitted and on 06/05/2025 we called yourself to book the 14/05/2025 for further BER listing, contents packing and removal including items inside kitchen base units. These works were completed on 14/05/2025.

* Most of the contents and materials were present in this environment for around three months, with some remaining for up to five months, until the final strip-out was completed toward the end of July 2025. - We spoke with yourself on 10/06/2025 whereupon you confirmed that the strip outs were to be completed that same day and we booked 12/06/2025 for the initial installation of drying however this was not installed as sanitisation was to be completed prior to this. We arranged to attend the property on 14/06/2025 to undertake the sanitisation, ATP swabbing and installation of drying equipment.

* Throughout this period, the house was subject to persistent dampness and high humidity, consistent with long-term Category 3 (sewage) contamination. - Agreed

Whilst we acknowledge that a period of drying and treatment was undertaken following the first strip-out, this was stopped when it became clear that a further strip-out would be required. No drying or dehumidification has been undertaken following the second strip-out, to the best of my knowledge. During both strip-outs, there was an evident stench of sewage and high levels of moisture were apparent, conditions which the builders are able to attest to. - Following my visit of 09/07/2025 where the additional requirement for stripping out was noted drying was ceased as whilst the majority of the property was deemed to be at acceptable levels the floating floor construct was deemed as needing removal. We then awaited the stripping out works. Additional strip out works were noted as being completed on during the week commencing 04/08/2025 following which we made arrangements to attend on 06/08/2025 for further sanitisation and drying re-installation and as such drying was undertaken following the second strip out works. Drying equipment was removed on 28/08/2025. Please note during all of the above dates for drying moisture checks were undertaken.

At our most recent meeting with our expert, Mr Adam Clutterbuck, and the insurance surveyor, it was observed by Mr Clutterbuck that “visible wet areas were evident along the floor-slab edges and at joints between sections of the slab.” This observation was made during a prolonged dry period and therefore indicates ongoing moisture ingress within the building fabric, providing continuing conditions for microbial persistence and activity. - Given the properties age, construction and location historical issues are likley to be impacting moisture levels within the property, however further investigation into the areas of concern would need to be undertaken to identify causation.

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2. Medical Backdrop (recorded for context, not for agreement)

It is relevant to record the professional opinion of Dr Vanya Gant, Consultant Physician in Cellular Immunology and Infectious Diseases, who has been directly involved in Mrs Louisa Scarr's care. Dr Gant has confirmed that:

* Mrs Scarr is clinically vulnerable, having undergone major surgery, radiotherapy for breast cancer, and now suffering from advanced chronic kidney disease while carrying a multidrug-resistant Gram-negative organism.

* This combination places her at a significantly heightened risk of both bacterial and fungal infection, the latter occurring through inhalation of airborne mould spores which may also trigger hypersensitivity reactions.

* Dr Gant describes sewage as "one of the richest sources of both bacteria and moulds known to man," noting that residual dampness will hugely increase airborne mould levels and that it is therefore "very important for the preservation of Mrs Scarr's future health to make every effort to reverse the totality of the sewage-related damage … including double deep cleaning to hospital standards."

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3. Airborne Contamination and Implications for Materials and Equipment

The period during which the property and its contents remained in a sewage-contaminated, high-humidity environment resulted in airborne dissemination of microbial material throughout both floors of the house.

Porous substrates — particularly kitchen units, timber joinery, plaster surfaces, insulation, soft furnishings, wooden furniture, and canvases — are prone to absorbing and retaining these airborne contaminants.

In addition, electrically ventilated or air-handling appliances — including extractor fans, cooker hoods, washing machines, dishwashers, consumer units, refrigerators, Everhot cookers, and televisions with cooling vents — would have drawn contaminated air directly through their housings and internal components. These systems cannot be effectively cleaned or validated as sterile.

While in theory certain forms of specialist sterilisation exist for industrial or laboratory-grade equipment, such processes are entirely impractical and economically prohibitive in a domestic context. No commercially realistic method exists to decontaminate domestic appliances to a standard approaching hospital sterility.

Given the nature and duration of the exposure, these materials and systems cannot be decontaminated to a standard consistent with hospital-grade sterility and must therefore be removed and replaced. - Beyond sanitisation and ATP swabbing to the floor slab identification of airborne contamination was not requested.

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4. Points Confirmed During Our Meeting

During our discussion, you confirmed that:

* There is no economically viable method to verify the complete elimination of pathogens from porous or absorbent materials once exposed to sewage.- Agreed, whilst a number of processes exist for effective decontamination to be completed verification at the level you have requested would not be econimical in regards to the potential replacement cost of the items.

* Surface or ATP swab testing is limited to the exact area and time tested and cannot evidence systemic sterility.

* For completeness, I note that ATP surface testing is sometimes carried out in such situations; however, as was discussed, such testing can only ever provide a momentary indication at the precise point sampled and cannot demonstrate systemic sterility or the absence of embedded contamination. - I have included below the generally accepted Key limitations of using ATP swabbing to identify the sanitary status of an entire area include:

* Does not detect all contaminants: ATP testing measures adenosine triphosphate, an energy molecule found in all living and some once-living cells (like food residue, blood, or skin cells), as a broad indicator of organic matter. However, it does not directly detect specific pathogens, viruses (which are not living cells), or bacterial spores

* Cannot determine sterility: A "zero" RLU (Relative Light Unit) reading does not mean a surface is sterile; sterility is the complete absence of all microorganisms and their spores. An ATP test only indicates low levels of detectable organic residue.

* Interference from chemicals: Residual cleaning chemicals, particularly bleach solutions, can interfere with the bioluminescence reaction in the test, potentially leading to inaccurate or false-negative results even if the area is not clean.

* Variability in results: The results can be subject to inherent variability based on the specific ATP device brand, the user's swabbing technique (e.g., pressure, surface area sampled), and surface type (e.g., porous vs. non-porous materials). This imprecision can lead to a significant chance of a false reading with only a single swab sample.

* Sampling limitations: A single swab only assesses a very small, specific spot. The results from that single swab cannot be reliably extrapolated to confirm the cleanliness of an entire room or even a large surface, as contamination may be unevenly distributed in "hot spots".

* Does not differentiate "good" vs. "bad" ATP: The test detects all sources of ATP and cannot distinguish between harmless organic matter (e.g., a drop of juice) and potentially dangerous bacteria, or even the "good" bacteria used in probiotic cleaning products

* Any attempt to restore such materials to hospital standards would be scientifically unreliable outside the context of a laboratory sanitisation programme costing far in excess of any practical or economic threshold. - Agreed as per my previous comment above relating to economical viability of undertaking this for each individual item.

On that basis, and from a biotechnical perspective, it is clear that if the objective is to return the property to a hospital-grade standard, it would be impossible to disagree with the following statement:

* Porous and absorbent materials, including kitchen units, joinery, plasterboard/plaster, insulation, timbers, soft furnishings, wooden furniture, and canvases, must be removed and replaced.

* Electrically ventilated or air-handling appliances, including extractor fans, cooker hoods, washing machines, dishwashers, consumer units, refrigerators, Everhot cookers, and televisions with cooling vents, must also be replaced.

* Full internal redecoration is required throughout to address the prolonged airborne exposure and humidity affecting both ground and first floors. - Whilst i agree on the basis of the expectations of yourself as laid out above, a seperate conversation would need to take place regarding the requirement for hospital standards to be achieved and as i am neither a medical profesional nor microbiologist i would not comment further.

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5. Clarification of Shared Understanding

Many of the points above have already been rehearsed between us on several occasions, and I recognise that they already reflect BioResponse's own professional view. The purpose of this record is to remove any possible ambiguity about that position. Both Andy and I recall, for example, your remark that you were surprised the kitchen units had not already been classified as Beyond Economic Repair (BER). In the wider context of our discussions, the same principle clearly extends to all other categories listed. While the decision on replacement ultimately rests with insurers, this letter I trust stands as an accurate and fair record of the biotechnical position that we would both agree upon. - I did agree that the base units should be reviewed if ingression had taken place and should contamination/ingression be identified considerations for thier replacement given.

I would be grateful if you could confirm in writing that this summary reflects BioResponse's professional understanding of the points raised during our discussion. - PLEASE NOTE, my above comments relate to queries raised regarding your expectations taking into account economical viability, ultimately a discussion between insurers and yourself will need to take place for clarification amongst all relevant parties.

Many thanks Darren.

Nicholas

Nicholas Scarr

1 Riverside Cottages

AlresfordNicholas Scarr

CEO – Quantlogic

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Attachments (7):

- image001.png (128,288 bytes)
- image002.png (32,570 bytes)
- H&C Report [DRAFT 1] -1 Riverside Cottages, The Dean, Alresford, SO24 9BE -15-11-2025.pdf (12,102,978 bytes)
- Appendix 1 -PLANS_AND_ELEVATIONS-2018.pdf (401,809 bytes)
- Appendix 2 - Geology of The Dean.pdf (208,278 bytes)
- Appendix 3 - Position Paper Waterproofing - 1 Riverside Cottages The Dean Alresford SO24 9BE.pdf (336,597 bytes)
- Appendix 4 - Maclennan waterproofing and gas resistance proposals of 04-11-2025.pdf (1,131,616 bytes)